

ختم
شعار الجمهورية

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Name: مرفت احمد عبدالرازى عبدالمرول National ID: 27012110103921	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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A patient of thyroid gland tumor for Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
السلطة الوطنية للانتخابات

مرقت
احمد عبدالراضى عبدالمولى
١ ش محمد ابراهيم - ميت عقبة
العجوزة - الجيزة

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LWD642423

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٢٠٢٦/٠٧ ٢٧٠١٢١١٠١٠٣٩٢١

رؤية منزل

انتش مسلمة متروجة
عبدالكريم عبدالرحيم عبدالودود عبدالرحيم

البطاقة سارية حتى ٢٠٢٣/٠٧/١١

Patient's Name: Mervat Ahmed Abdel-Radi Abdelmawla
Date: 2026-08-29
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

REAL TIME NECK ULTRASOUND

□ Thyroid gland: [in normal site]

- The thyroid gland appears average sized with homogenous echotexture
- Average overall vascularity
 - o A solid hypoechoic nodule seen wider than tall with ill-defined margins and punctate echogenic foci. TIRAD 5

[AP x TD x CC]

- The right lobe measures 20 x 15 x 35 mm .
- The left lobe measures 16 x 12 x 32 mm.
- The isthmus measures 5 mm in height.
- No retro-sternal extension.
- Central trachea.

□ Few right upper cervical LNs largest seen globular measuring 1.6cm.

CONCLUSION:

- Right thyroid nodule (TIRAD 5) for FNAC.
- Right upper cervical suspicious L.N for further assessment.

Best regards,
DR. Sahl Mahmoud

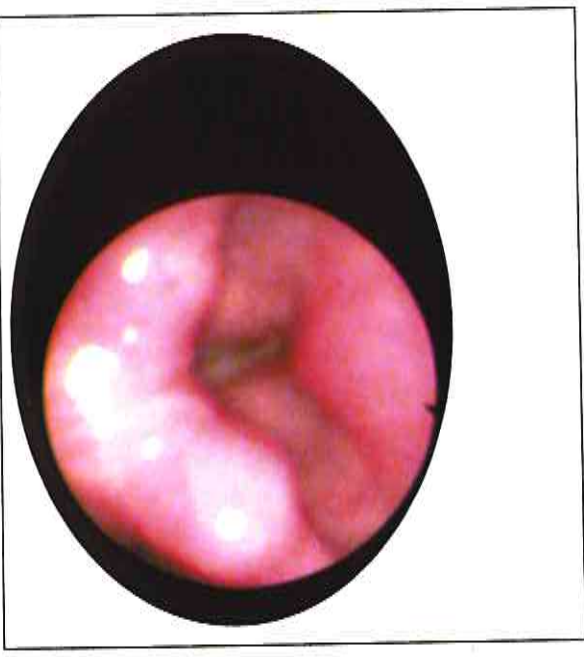
HEARING & SPEECH INSTITUTE

Patient Info

Name: Mervat Ahmed Abdelrade abdelmola
 Gender: female
 Date of Birth: 11/12/1970
 Complaint: pre-thyroidectomy
 Smoking: non smoker

Exam System Info - General

Occupation: house wife
 ID: 0103921
 Exam Original Date: 3 /10/2026
 Examining Facility: flexible telescope



Exam Summary:

- Supra glottic: both arytenoids are congested and edematous with interarytenoid corrugations, GERD??
- Ventricular hypertrophy
- Glottic: both vocal folds are mobile in adduction and abduction. There is 1mm gap during phonation
- Subglottic: seen part is normal .

Examiner:

Dr. Riham Alnny



Patient's Name:

Mervat Ahmed Abdel-Radi Abdelmawla

Date:

2026-08-29

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

REAL TIME NECK ULTRASOUND

Thyroid gland: [in normal site]

-The thyroid gland appears average sized with homogenous echotexture

-Average overall vascularity

- o A solid hypoechoic **node** seen wider than tall with ill-defined margins and punctate echogenic foci. TIRAD 5

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Gustave Roussy International-Egypt- Pathology Lab
جورستاف روسي - مصر - مختبر أمراض البطانة



Patient Name	مرثى احمد عبداللطفي	Case No.	361678
Sex/Age	Female 55 Y	Patient No.	26522
Registered Date	31/8/2026 12:44:33 PM	Referred By Dr.	Interventional Radiology
Validation Date	16/9/2026 04:46:37 PM	Branch	Gustave Roussy
Reporting Date	19/9/2026 02:07:48 PM	Pathology No.	C-214/26

Pathology Report

Clinical Data: Neck U/S showing right thyroid lobe nodule (TIRADS 5) and right upper cervical suspicious lymph node.

Nature of specimen: Right thyroid lobe nodule, U/S guided FNA, Cytology.

Gross specimen

Four Alcohol fixed & then (H&E) stained smears were received.

Microscopic examination

- Adequacy: Adequate smears.
- Cellularity: Cellular.
- Colloid: Present.
- Follicular cells: Present, forming papillary fronds and streaming sheets.
- Nuclear enlargement: Present.
- Nuclear grooving: Present.
- Nuclear pseudo-inclusions: Present.

Diagnosis

Right thyroid lobe nodule, U/S guided FNA, Cytology:

- Suspicious for malignancy (Suspicious for papillary thyroid carcinoma), Bethesda category V.

Comment: The BETHESDA System for reporting thyroid cytopathology: Diagnostic categories:

- Non-diagnostic or Unsatisfactory.
 - Benign. Consistent with a benign follicular nodule (includes adenomatoid nodule, colloid nodule, etc.).
- Consistent with chronic lymphocytic (Hashimoto) thyroiditis in the proper clinical context.
- Consistent with granulomatous (subacute) thyroiditis.



خبر رقم ٢٤ رقم (4) - (٩٨٧) - مختبر أمراض البطانة - مصر - مختبر أمراض البطانة - مصر
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Gustave Roussy International-Egypt- Pathology Lab
جوستاف رويسى انترناتيونال - مصر-معمل الباثولوجى



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- III. Atypia of undetermined significance or follicular lesion of determined significance.
- IV. Follicular neoplasm or suspicious for follicular neoplasm.
- V. Suspicious for malignancy.
- VI. Malignant.

Signature
Prof. Dr. Maha Guimei, MD, PhD.



غرفة رقم (4) - طور التول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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لدعم صحة المرأة المصريةصحة
مصر
Ministry of Health
Egypt

09/27/2026

MDT FORM

Name:	مراف احمد عبدالغنى عبدالغنى	MRN:	12990
Age:	56 Y	ID:	27012110103921
Menopausal Sta:	post	Phone:*	01222367334
NRS SCORE:		Marital Status:	married
BMT:		Family History:	-ve
Gender		Reason Of MDT:	new
CO Morbidity:	dm	ECOG Performance Status	☐ 0 ☒ 1 ☐ 2 ☐ 3 ☐ 4
Surgical History:*	cs		

Clinical Data:

BX:

Suspectious for malignancy (papillary thyroid carcinoma)

ER:

PR: HER2: KI67:

History:

ptn gave hx of ICU admission with pericardial effusion and pneumonia then discover rt thyroid nodule ptn has hoarsness of voice (Laryngoscope requested)

Sonomammography:

neck us rt thyroid nodule (TIRAD 5)

rt upper cervical suspicious LN

Biomarker/Molecular

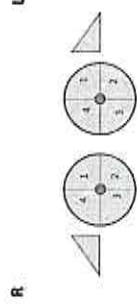
Mets Workup:

Recommendations:

MDT Outcome & Plan

FOR FNAC from cervical LN

total thyroidectomy +/- neck dissection



Right Side

Left Side

Q1 Q2 Q3 Q4 Q1 Q2 Q3 Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. حنان عزت	د. احمد لطيف		

about:blank